

Pharmaniaga Chlorpheniramine Tablet

DESCRIPTION

Yellow, round, flat, bevelled-edge tablet, with Pharmaniaga icon on one side and scored on the other.

COMPOSITION

Each tablet contains Chlorpheniramine maleate 4 mg.

ACTIONS

Chlorpheniramine maleate is competitively antagonises the effect of histamine on capillary permeability and on the bronchial, vascular and many other types of smooth muscle.

Chlorpheniramine maleate is a H1-receptor antagonist.

a) Effects on the smooth muscle:

It inhibits the constrictor action of histamine on the respiratory smooth muscle. Within the vascular tree, it inhibits both the vasoconstrictor effects of histamine as well as vasodilator effects.

b) Effects on capillary permeability:

Chlorpheniramine strongly antagonises the action of histamine that result in increased capillary permeability and formation of oedema.

c) Effects on the CNS system:

In the usual therapeutic doses, chlorpheniramine causes the specific kinds of CNS depression or sedation, but to some patients, even the therapeutic dose can stimulate the CNS and cause restlessness and hyperactivity. However, the CNS stimulation usually occurs at the acute poisoning state.

d) Effects on cardiovascular system:

Rapid intravenous injection of chlorpheniramine will cause a transient fall in blood pressure, probably related to local anaesthetic activity.

e) Absorption, Fate and Excretion:

Chlorpheniramine is rapidly absorbed from the G.I.T., metabolised in the liver, and excreted mainly as metabolites in the urine. Following oral administrations the effects start within 15 to 30 mins and are fully developed within 1 hour and last for about 4 – 6 hours. The metabolites are present in the plasma for about 48 hours. About 34% of the dose will be excreted in the faeces.

INDICATIONS

Allergic conditions which are responsive to antihistamines such as hay fever, urticaria and food allergies.

CONTRAINDICATIONS

Patients with lower respiratory disease, including asthma. Patients who have been treated with MAOIs, within the last 14 days. Patients sensitive to antihistamines.

ADVERSE REACTIONS/ SIDE EFFECTS

CNS Depression:

Drowsiness, lassitude, inability to concentrate, fatigue, dizziness, insomnia, irritability, tremor. Nightmare, hallucinations and euphoria have occasionally been reported. In over intoxication CNS stimulation may lead to convulsions, vertigo, tinnitus, dilated pupils, labyrinthitis. Others include dry mouth, nose and throat, due to an anticholinergic affect.

Haematological:

Blood dyscrasias, agranulocytosis, haemolytic anaemia, leucopenia, thrombocytopenia, although rare, have been reported.

Genitourinary:

Voiding difficulties, dysuria and frequency retention.

Respiratory:

Thickening of bronchial secretions, tightness in chest, wheezing, nasal stuffiness.

General:

Hypothermia, anaphylactic shock, palpitation, biliary spasm, photosensitivity, excessive perspiration, sweating.

Antihistamines sometimes cause rashes and hypersensitivity reactions (including bronchospasm, angioedema and anaphylaxis) and cross sensitivity to related drugs may occur.

Cardiovascular:

Orthostatic hypotension, tachycardia, hypotension, headache, and palpitations ECG changes. Overdosage can lead to cardiovascular collapse.

Gastrointestinal:

Nausea, vomiting, diarrhoea.

PRECAUTIONS

Pregnancy:

Studies in humans have not been done, Studies in animals have not shown that these medicines cause adverse effects in the foetus.

Lactation:

This drug may inhibits lactation because of the anticholinergic actions. Small amount of the drugs are distributed into breast milk. Use is not recommended in nursing mothers because of the risk of adverse

effects, such as unusual excitement or irritability, in infants.

Paediatrics:

In infants and children especially, antihistamines in overdosage may cause hallucinations, convulsions or death. Antihistamines in young children particularly may produce excitation. Use is not recommended in new born or premature infants because this age group has an increased susceptibility to anticholinergic side effects, such as central nervous system (CNS) excitation and an increased tendency towards convulsions.

Geriatrics:

Use in geriatric patients may be more likely to cause dizziness, sedation, confusion, hypotension and a paradoxical reaction characterised by hyperexcitability. Medication should be discontinued if the anticholinergic side effects, such as dryness of mouth and urinary retention (especially in males), of the antihistamine, continue to occur or are severe, which are especially susceptible to geriatric patients.

Use with caution in narrow angle glaucoma, stenosis peptic ulcer, pyloroduodenal obstruction, symptomatic prostatic hypertrophy and bladder neck obstruction.

Antihistamine should not be used to treat lower respiratory tract symptoms especially asthma.

May precipitate fits in epileptic, in large doses of antihistamines. Elderly patients (60 years or older) – more likely to cause dizziness, sedation and hypotension.

WARNING:

i) Not suitable to be used for children below 2 years of age.

ii) Use carefully and follow the advice from doctor or pharmacist for children aged 2 to 6 years.

DRUG INTERACTIONS

Alcohol or CNS depression – producing medications- Concurrent use may potentiate the CNS depressant effects of either these medications.

Anticholinergic drugs – Anticholinergics effects, such as paralytic ileus, may be potentiated when these medications are used concurrently with antihistamines.

Monoamine oxidase inhibitors – Concurrent use of MOA inhibitors with antihistamines may prolong and intensify the anticholinergic and CNS depressant effects of antihistamines. Concurrent use is not recommended.

Ototoxic medications – Concurrent use may mask the symptoms of ototoxicity such as tinnitus, dizziness or vertigo.

DOSAGE AND ADMINISTRATION

Adults and children over 12 years	:	4 mg t.d.s or q.d.s.
Children 6 – 12 years	:	2 mg t.d.s or q.d.s. or as directed by the physician.
Children 2 – 5 years	:	1 mg every 4 – 6 hours (maximum 6 mg daily)

ROUTE OF ADMINISTRATION

Oral

OVERDOSAGE

Symptoms:

In small children:

Hallucinations, excitement, ataxia, flushed face and fever. For serious case, there is a deepening coma with cardiorespiratory collapse and death.

In adults:

Fever, flushed face, excitement leading to convulsions and postictal depression preceded by drowsiness and coma.

Treatment:

Mechanical support of ventilation. If convulsions develop, they are best countered by a short acting depressants such as thiopental.

STORAGE CONDITIONS

Store below 30°C.

Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In blisters of 100, 500 and 1000 tablets.

Date of revision: 28th May 2024

PRODUCT	REGISTRATION	HOLDER/
MANUFACTURER:		
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